Axcel®

IBUPROFEN PAEDIATRIC SYRUP

MAL20002268AZ

COMPOSITION:
Each 5ml contains
Ibuprofen 100mg
Sodium Benzoate 0.2% w/v as preservative.

PRESENTATION:
Orange color and orange flavoured syrup.

INDICATIONS:
Relief of fever such as fever associated with cold and flu; and pain such as pain from teething and toothache, earache, sore throats, headache and minor aches and sprains. Also indicated for the relief of inflammation of muscles and joints.

PHARMACOLOGY
Mechanism of Action:
Ibuprofen, a phenylpropionic derivative, a non-steroidal anti-inflammatory agents which also posses analgesic and antipyretic effects. It is an inhibitor of cyclo-oxygenase. The precise mechanisms of action is not known but it is thought that ibuprofen produces anti-inflammatory effect by inhibiting prostaglandin synthesis.

Pharmacokinetics:
Ibuprofen is well absorbed from the gastrointestinal tract after oral administration. Peak plasma concentrations occur about 1 to 2 hours after ingestion. It has a half-life of about 2 hours. Ibuprofen is 90 to 99% bound to protein plasma.
Kidney is the major route of excretion where ibuprofen is excreted as metabolites and their conjugates. About 1% is excreted in the urine as unchanged ibuprofen and about 14% as conjugated ibuprofen. There appears to be little if any excretion in breast milk.

DOSAGE & ADMINISTRATION:
Oral administration.
SHAKE WELL BEFORE USED.
The daily dosage is 20-40 mg/kg body weight in divided doses.
Baby under 6 months old initially 2.5ml (50mg) followed by 2.5ml (50mg) 6 hours later if required, not more than 5ml (100mg) in 24 hour.
Babies 6-12 months : 2.5ml (50mg) three to four times a day.
Children :
1-3 years : 5ml (100mg) three times a day
4-6 years : 7.5ml (150mg) three times a day
7-9 years : 10ml (200mg) three times a day
10-12 years : 15ml (300mg) three times a day

CONTRAINDICATION:
Contraindicated in patients with peptic ulceration and known hypersensitivity to ibuprofen, other-non-steroidal anti-inflammatory drugs (NSAIDs) or aspirin.

PRECAUTION:
Ibuprofen should be given with care to patients with asthma or bronchospasm, bleeding disorder, a history of peptic ulceration, a history of hypersensitivity to aspirin or other NSAID, hypertension, and impaired renal, hepatic or cardiac function, and systemic lupus erythematosus (SLE). Ibuprofen should be discontinued in patient who experienced blurred or diminished vision. Patients with collagen disease may be at greater risk of developing aseptic meningitis.

Cardiovascular Thrombotic Events:
Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAIDs, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration. There is no consistent evidence that the concurrent use of aspirin mitigate the possible increased risk of serious cardiovascular thrombotic events associated with NSAIDs use.

Hypertension:
NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAIDs treatment and at regular intervals thereafter.

Heart Failure:
Fluid retention and oedema have been observed in some patients taking NSAIDs, there caution is advised in patients with fluid retention or heart failure.

Gastrointestinal Events:
All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patient about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.

Severe Skin Reactions:
NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

WARNINGS
RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAIDs

Serious GI toxicity such as bleeding, ulceration and perforation can occur at anytime, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.
Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding.
Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilatated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for GI events.

Use in Pregnancy and Lactation:
Regular use of ibuprofen during the 3rd trimester of pregnancy may result in closure of foetal ductus arteriosus in utero, and possibly in persistent pulmonary hypertension of the newborn. The onset of labor may be delayed and its duration increased. There is no report on the use of ibuprofen during lactation. Use of ibuprofen during pregnancy and lactation should be avoided if possible.

SIDE EFFECTS:
The most frequent adverse effects are gastrointestinal disturbance such as abdominal discomfort, nausea and vomiting, abdominal pain, gastrointestinal bleeding or activation of peptic ulcer. Other frequent effects involving CNS include headache, dizziness, nervousness, tinnitus, depression, drowsiness and insomnia. Hypersensitivity reactions may occur occasionally and include fever, asthma, and rashes. Hepatotoxicity, aseptic meningitis and some patients may experience visual disturbances. Less frequent side effects include agranulocytosis, aplastic anaemia, thrombocytopenia, toxic amblyopia, acute renal failure, fluid retention and nephrotic syndrome.

DRUG INTERACTIONS:
Possible enhancement of the effects of oral anticoagulants and increased plasma concentrations of lithium, methotrexate, and cardiac gylcosides. The antihypertensive effect of ACE inhibitors, betablocker, and diuretics may be reduced. Convulsions may occur due to an interaction with quinolones. Enhance the effects of phenytoin and sulphonylurea antidiabetics. The concomitant use of more than one NSAID should be avoided because of the increased of adverse effects. Risk of gastrointestinal bleeding and ulceration associated with ibuprofen is increased when used with corticosteroids.

OVERDOSAGE & TREATMENT:
A syndrome of coma, hyperkalaemia with cardiac arrhythmias, metabolic acidosis, pyrexia, and respiratory failure was reported upon the overdosage of ibuprofen. As for the treatment, evacuate the stomach contents by emesis if the patients is alert. If he/she is depressed, use air way protected gastric lavage. Absorption of the remaining ibuprofen can be delayed by giving activated charcoal.
Since the drug is acidic and excreted in the urine, the alkalisation of the urine may promote more rapid excretion. The supportive and symptomatic measures / treatment should be instituted whenever necessary.

STORAGE:
Keep container well closed. Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN
JAUHI DARI KANAK-KANAK

PACK QUANTITIES:
Available in 60ml & 100ml PET bottle.

Further information can be obtained from pharmacist, physician or the manufacturer.

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